Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Submitted electronically via www.regulations.gov

RE: Comments to Health Technology Ecosystem Request for Information (CMS-0042-NC)

Dear Administrator:

I am Anish Sebastian, Co-founder and CEO of 1EQ Inc. ("**Babyscripts**"). I am joined in this letter by Berkley Nelson, Head of Reimbursement and Commercial Operations for Babyscripts. We want to take this opportunity to provide feedback in response to the Request for Information ("**RFI**") issued by the Centers for Medicare & Medicaid Services ("**CMS**") and the Assistant Secretary for Technology Policy ("**ASTP**")/Office of the National Coordinator for Health Information Technology ("**ONC**") (collectively, "**ASTP/ONC**") regarding the advancement of a patient-centric digital health ecosystem. **As a general matter, we appreciate CMS' ongoing efforts to foster innovation, reduce barriers to data access, and align digital health policy with the goals of value-based care.**

Babyscripts is a digital health company that provides a comprehensive maternity care program that spans the prenatal and postpartum periods to help providers and payors ("**Customers**") improve health outcomes for their patients and members. As part of this program, Babyscripts has developed a remote physiologic monitoring ("**RPM**") solution that Customers can use to monitor and manage the risk of complications relating to hypertension in pregnancy (i.e., preeclampsia, eclampsia, HELLP syndrome), diabetes (i.e., gestational diabetes), and behavioral health. Babyscripts also aims to help improve pregnancy outcomes and prevent maternal and fetal mortality and morbidity.

In this comment letter, Babyscripts will provide supporting information for each of the following key points:

<u>Summary of Key Points:</u>

- We suggest that our RPM solution, Babyscripts, could serve as a health management, care navigation, and personal health record app that could be useful to Medicare and

Medicaid beneficiaries and their caregivers.

- We urge CMS to permit providers to bill an RPM device/supply code for each device supplied per month (instead of allowing billing/reimbursement for only one device).
- We urge CMS to update the RPM and remote therapeutic monitoring ("**RTM**") device/supply code reimbursement rates to reflect current technology costs.
- We encourage the removal of patient deductibles for RPM services, which could improve patient access to RPM and streamline provider workflows.
- The interoperability challenges encountered by value-based care RPM programs include integrating remote patient data with electronic health record ("**EHR**") systems and achieving consistency among data formats.
- Value-based care standards should shift towards outcome-based criteria.

Please find our responses below to CMS' questions posed in this RFI:

a. **Question PC-3.** *Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?*

Our RPM solution, Babyscripts, is a health management, care navigation, and personal health record app that could be useful to Medicare and Medicaid beneficiaries and their caregivers.

Babyscripts is a digital health platform designed to enhance maternal care through RPM, education, and care coordination. The platform primarily focuses on pregnancy and postpartum care and can be beneficial to Medicare and Medicaid beneficiaries and their caregivers, particularly those managing chronic conditions or requiring ongoing health monitoring.

The Babyscripts RPM program is designed with a maternal health focus. The underlying design principles and functionalities have broad applicability. For example, our platform integrates RPM with personalized patient education, robust data capture, and secure, asynchronous communication with care teams. These features mirror what you see in other digital health management apps which allow users to easily track their health data, schedule appointments, and receive timely updates on their care plans.

Moreover, health management apps like Babyscripts that support remote monitoring routinely incorporate elements like real-time biometric tracking, automated alerts for out-

of-range values, and even care navigation tools to help patients understand when and how to seek care. This comprehensive approach can be especially helpful for beneficiaries managing chronic conditions or coordinating care among multiple providers. In fact, functionalities such as secure personal health records, integrated education modules, and proactive patient engagement have been central to our strategy and are precisely the attributes that make digital health apps so effective in managing complex care regimens.

b. **Question PR-1.** *What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients? What are the current obstacles?*

We urge CMS to permit providers to bill the RPM device/supply codes for each device supplied per month instead of only allowing providers to bill one RPM device/supply code per month even when multiple devices are being used as part of a patient's RPM treatment plan.

Currently, regardless of how many eligible RPM devices a patient is using, the specific device/supply codes associated with those services (CPT code 99454) can only be billed once per patient within a 30-day period. That is, a provider monitoring multiple medically necessary RPM devices a month for a single patient can only bill for one unit of CPT code 99454 per month, which may not sufficiently cover the value or cost of multiple devices. This may disincentivize providers from providing effective care management to patients who may significantly benefit from these services.

For example, at Babyscripts, we often work with providers whose patients suffer from chronic, uncontrolled, or gestational-induced hypertension with the onset of gestational diabetes. Therefore, the provider may find it medically necessary to order a blood pressure cuff for hypertension monitoring *and*, simultaneously, a blood glucose meter ("**BGM**") or continuous glucose meter ("**CGM**") for diabetes monitoring. However, the provider can only bill for supplying one of those devices, and the costs associated with ordering multiple devices for a single patient often outweigh the reimbursement associated with a single unit of CPT code 99454. For example, the combined cost of a blood pressure cuff and a BGM can exceed the current reimbursement rate for a single RPM session/episode of care. Therefore, the aggregate cost of multiple medically necessary devices may hinder a provider's ability to be able to implement comprehensive remote monitoring. As explained in further detail below, this restriction limits access to comprehensive digital health services and minimizes the full utility of RPM services.

To provide the highest quality patient care, providers are ordering multiple devices for patients in a given month, when medically necessary, even though they are not receiving reimbursement for more than one device. Bundling the devices from a reimbursement perspective undervalues and minimizes the care management services provided. Providers should be able to bill a device/supply code for each monitoring device that their patient uses as there is an additional cost to providing more than one device to a patient.

Further, only allowing one device code to be billed per month discourages care coordination and effective care management. If providers can only receive reimbursement for one device, then there is less incentive to provide patients with other devices for their conditions that may need monitoring. There is no reimbursement to cover the costs of additional devices, which can include device sourcing, onboarding, software, etc. Limiting providers' device use to what is reimbursed is a direct hindrance to comprehensive care management.

Permitting providers to bill device/supply codes for each medically necessary device supplied to a patient will encourage providers to leverage all necessary digital health products for their patients, allowing for effective care management and overall improved patient outcomes.

We urge CMS to update the RPM and RTM device/supply code reimbursement rates to reflect current technology costs.

The RPM and RTM device/supply codes (CPT code 99454 for RPM, and CPT codes 98976, 98977, and 98978 for RTM) are practice expense ("**PE**")-only codes, and the relative value units ("**RVU**") for these services are determined by adding the cost of direct resources, medical supplies, and medical equipment. The medical device supplied to the patient is considered equipment and is a direct PE input for these codes. The current reimbursement rates are based on data from 2007-2008, which precedes the widespread adoption of many current digital therapy and monitoring technologies. Thus, the current reimbursement rates do not accurately account for the costs of innovative and emerging technologies.

Recent RPM/RTM devices are much more sophisticated than a basic manual blood pressure cuff, which is presumably what CMS contemplated when initially determining the reimbursement methodology associated with these codes. Many devices in the market today used for RPM/RTM include cloud integration, cellular/Bluetooth connectivity, and enhanced patient engagement features. These capabilities carry higher costs than is

reflected in the current reimbursement rates. Because these costs are not reflected in the reimbursement rates, providers are disincentivized from employing devices that meet higher technical standards, which in turn limits patient access to the most effective tools to manage their health.

For example, the cost of an FDA-cleared cellular blood pressure cuff with direct EHR integration can range from $70-80 per patient per month. The cost of a cellular-enabled blood pressure monitor with automatic data transmission can range from $70-120 per patient per month. Per CMS, the non-facility national payment amount for 99454, 98976, and 98977 is $43.02.

Updating the reimbursement rates for RPM and RTM device codes would reflect the real-world cost of current digital therapy and monitoring technologies and would encourage providers to leverage approved digital health products for their patients.

c. **Question PR-2.** *What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?*

We encourage the removal of patient deductibles for RPM services, which could improve patient access to RPM and streamline provider workflows.

Removing or revisiting applicable deductibles for RPM services could lead to a dual benefit: increased patient access due to reduced out-of-pocket costs and enhanced operational efficiency in provider workflows. In turn, this may spur further innovation in RPM technologies by allowing developers to integrate sophisticated features without the constraints of complex revenue cycle management and billing intricacies.

Removing deductibles for RPM services could have a transformative impact on patient access. High deductibles often serve as a barrier, discouraging patients from participating in RPM programs—even when these services offer substantial benefits in managing chronic conditions or preventing hospital readmissions. Without the cost deterrent imposed by deductibles, RPM services become more accessible, thereby encouraging enrollment among Medicare beneficiaries, especially those enrolled in high-deductible health plans ("**HDHPs**"). Furthermore, eliminating the need to collect deductibles can streamline physician workflows by reducing administrative overhead as it relates to verifying and monitoring patient deductible collections. This financial simplification would

not only enhance the patient experience but would also allow healthcare providers to focus more on delivering quality care and less on navigating complex billing challenges.

d. **Question VB-11.** *What specific interoperability challenges have you encountered in implementing value-based care programs?*

The interoperability challenges encountered by value-based care RPM programs include integrating remote patient data with EHR systems and achieving consistency among data formats.

When implementing value-based care programs with RPM, one of the most significant interoperability challenges lies in integrating continuous streams of remote patient data with existing EHR systems. Many healthcare organizations still rely on legacy systems not designed for real-time, bi-directional data exchange. This disconnect forces clinicians to manually reconcile RPM data with patient charts, adding administrative burdens and increasing the potential for errors in patient care and billing processes.

Another key interoperability issue is achieving semantic consistency across diverse systems. RPM devices generate extensive, often unstructured data that must be standardized for clear, actionable insights. In practice, differences in data formats and adherence to standards like Health Level Seven ("**HL7**") or Fast Healthcare Interoperability Resources ("**FHIR**") can lead to fragmentation—where critical information is misinterpreted or lost during transmission between disparate systems. This misalignment hinders the ability to track quality metrics accurately and coordinate care effectively, both of which are cornerstones of value-based care models.

Overcoming these challenges requires a collaborative approach—one that involves standardizing data formats and enhancing EHR-RPM integration. Such efforts not only streamline clinician workflows and reduce administrative overhead but also improve the accuracy of reimbursements tied to quality metrics. Addressing these issues is critical for leveraging RPM's full potential, ultimately driving improved patient outcomes and more efficient, cost-effective care delivery.

e. **Question VB.13.** *What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?*

Value-based care standards should shift towards outcome-based criteria.

Rather than relying heavily on process and structural metrics, shifting the focus toward outcome-based criteria can better align RPM programs with value-based care objectives. By developing standardized metrics that link RPM data directly to measurable clinical improvements—such as reduced readmission rates, better chronic disease management, and improved patient engagement—providers can document value without excessive administrative overhead. In practice, this means redefining quality measurement frameworks to reward tangible improvements and streamline the documentation process to capture only clinically meaningful data.

Standards and criteria should be continuously updated to clearly delineate which RPM activities are eligible for value-based reimbursement. Clear, evidence-based guidance on what constitutes a reimbursable RPM service—along with streamlined processes for documentation and audit trails—can alleviate ambiguity. When criteria support automated data capture and validation, providers are less burdened by regulatory complexity and can focus on delivering quality care that meets value-based benchmarks.

Conclusion

We appreciate CMS' efforts to foster innovation and align digital health policy with the goals of value-based care. To support these goals, we recommend that CMS consider policy updates that reflect the realities of current RPM implementation. Specifically, we believe that our RPM solution, Babyscripts, can serve as an effective digital health tool for health management, care navigation, and personal health records, particularly for Medicare and Medicaid beneficiaries and their caregivers. We urge CMS to allow providers to bill for each RPM device supplied per month, rather than limiting billing/reimbursement to a single device, and to update reimbursement rates for RPM and RTM supply codes to align with modern technology costs. Additionally, we encourage the removal of patient deductibles for RPM services, which could both improve access and streamline provider workflows. To fully realize the potential of RPM in value-based care, it is necessary to address interoperability challenges with EHR systems and to promote standardization across data formats. Finally, we believe that value-based care standards should focus more explicitly on outcome-based measures. In sum, addressing these challenges would collectively improve the RPM program and encourage utilization of these medically necessary services.

Thank you for your consideration of these comments.

Sincerely,

Anish Sebastian

Co-founder and CEO, Babyscripts